CATH PLAN · Case 3

59M, 102 kg · scheduled 10:00 · diagnostic cath ± PCI (exertional chest pressure) · pathway: indeterminate, pre-cath plan

Synthetic teaching content. Not clinical advice. Not a medical device. DRAFT for the chair to rule on.

PRE-CATH PLAN: angiography has not been done. The likelihood is a label from a proposed points rule, not a probability, and the packet lists equipment for both branches.

Thresholds: PROPOSED values, not yet verified by the chair. Template logic (likelihood weights and bands, prep rules, Imaging triggers, Heart Team factors, stage assignment, Labs' eGFR cutoff) is also unreviewed.

7	n/a	higher	1
flags from 4 agents	contrast ratio (published cutoff 3.7, Laskey)	likelihood band (9 pts, proposed rule)	devices out of stock

Contrast ratio = planned contrast (mL) ÷ eGFR. The published ratios use creatinine clearance, not eGFR (docs/EVIDENCE.md E1, E2): risk rose from 2 (Gurm 2011) and 3.7 was a discriminating cutoff (Laskey 2007). No guideline sets a limit; Gurm: risk rose from 2 and was clearest at 3 or more (EVIDENCE E1 does not record an advised limit); Laskey 3.7 is a discriminating cutoff, not a threshold for harm. The chair confirms the basis.

Pre-cath findings (CCTA, stress, symptoms)

CCTA lesion	Vessel	Stenosis	Calcium
L1	mid-LAD	75%	calcium none
L2	prox-LCx	55%	calcium moderate

Stress test: positive stress echo, anterior wall · EF: 55% · Symptoms: typical angina

Likelihood driver (proposed points rule)	Points
CCTA 75%, severe stenosis, CAD-RADS 4A/4B range (L1 mid-LAD; E5)	+2
CCTA 55%, intermediate stenosis (L2 prox-LCx; E6)	+1
two or more CCTA lesions	+1
stress test positive (stress echo, anterior wall)	+2
typical angina	+1
the positive stress result (anterior wall) is in the territory of the CCTA lesion	+2
Total · band: higher	**9**

A label from a points rule, not a probability. The chair decides whether to go on to PCI.

Lesion map (Imaging)

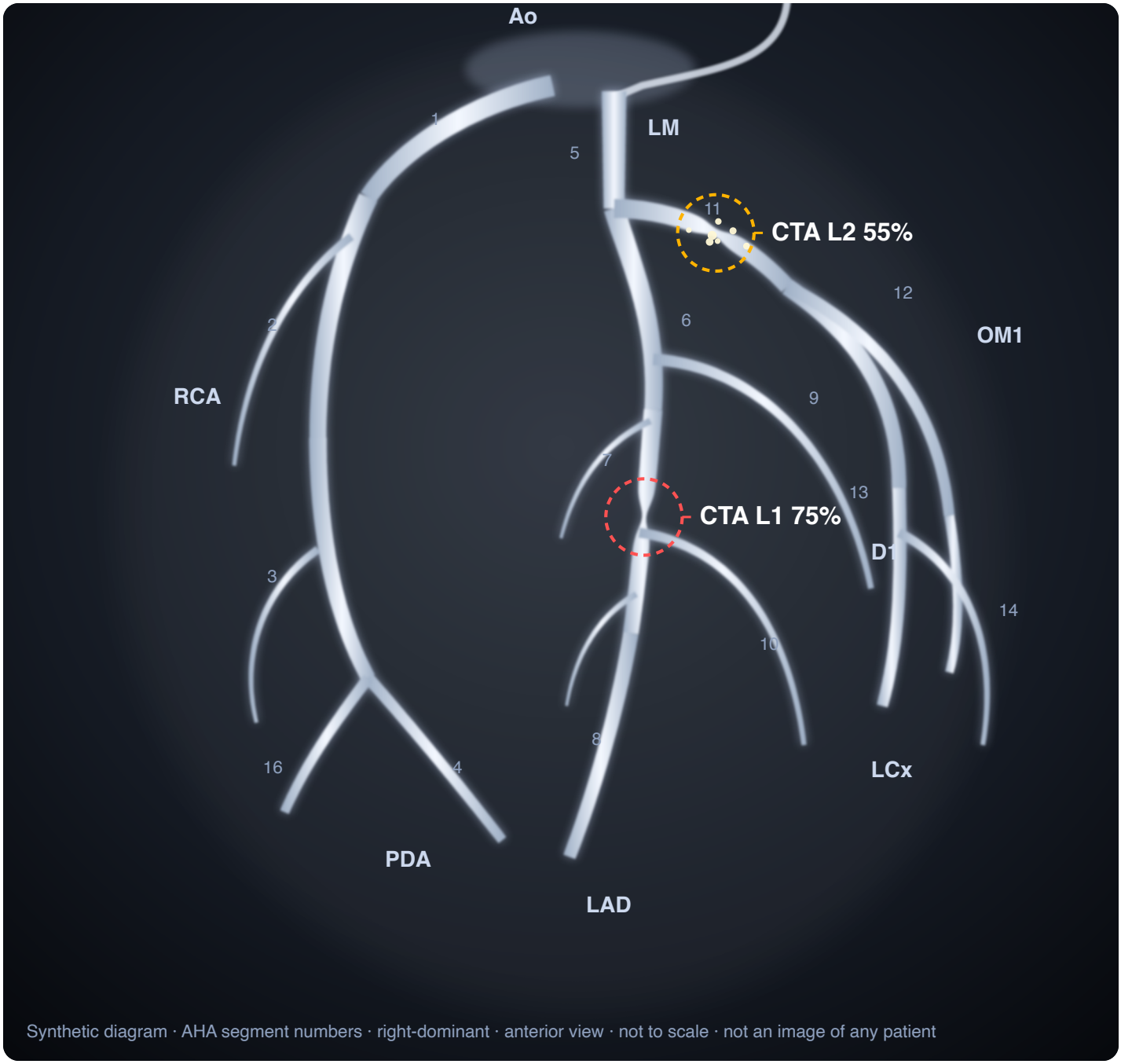
Lesion	Vessel	Stenosis	TIMI	Calcium	Flag
Angiography not yet performed: this is a pre-cath plan					

Angio findings: not assessed (angiography not yet performed). · Cath report: not in packet

Rules that fired

- Likelihood band (proposed rule, 9 points): higher. A label from a points rule, not a probability. Chair to decide; the packet lists equipment for both branches below.

Pre-cath CCTA findings (synthetic, not yet confirmed by angiography)



L1 · mid-LAD (AHA segment 7)
CCTA estimate 75% · calcium none · not yet confirmed by angiography

L2 · prox-LCx (AHA segment 11)
CCTA estimate 55% · calcium moderate · not yet confirmed by angiography

Drawn from the angio lines in the packet by tools/angio.py: right-dominant tree, anterior view, AHA segment numbers, lumen narrowed to the stated percentage, calcium shown as bright specks, distal filling dimmed when TIMI flow is below 3. Solid red ring = stenosis of 70% or more; dashed amber ring = under 70%, or a CCTA estimate not yet confirmed. A Medina 1,1,1 flag is drawn as a narrowing of about half the proximal LAD and LCx. A teaching diagram, not an image of any patient and not to scale.

Equipment manifest

Branch A · stays diagnostic	
Item (from the packet)	**Stock**
5F diagnostic catheters	8 on hand

Branch B · proceeds to PCI	
Item (from the packet)	**Stock**
6F guide	4 on hand
NC balloon 3.0 mm	2 on hand
DES 3.0x18 mm	**OUT: 0 on hand · no resupply scheduled**
standard PCI tray	4 on hand

Prep check (devices against findings)

Scope	The packet lists Branch A and Branch B (band higher, proposed rule)
Stock	**OUT: DES 3.0x18 mm**

On the shelf, by class (from the STOCK line)	
Guide catheters	6F guide 4, 7F guide 2
Diagnostic catheters	5F diagnostic catheters 8
Wires	support wire 4
Balloons	NC balloon 3.0 mm 2, NC balloon 3.5 mm 2
Stents (DES)	DES 3.5x24 mm 3, DES 3.5x28 mm 2, DES 3.0x18 mm 0 (no resupply scheduled)
Intravascular imaging	IVUS catheter 1
Support devices	Impella CP 0 (no resupply scheduled)

Questions for the chair (the plan asks, it does not choose a device):

- 2 lesions on CCTA and 1 stent named in the packet: chair to confirm what to stage.
- Calcium moderate on L2 prox-LCx (CCTA): the packet names no calcium-modification device or intravascular imaging catheter. Chair to decide whether any is needed.
- No wire is named in the packet's device lines: chair to confirm.

Flags from the agents

Imaging
- CCTA 75% · L1 mid-LAD · ref: CAD-RADS 2.0
- CCTA 55% · L2 prox-LCx · ref: ESC 2024 intermediate CCTA
- stress positive · anterior wall

Labs
- Hgb 9.8 vs ref 13.5-17.5
- INR 2.6 vs ref 0.9-1.1

Meds
- warfarin last dose 14 hours before the 06:30 report · diagnostic cath ± PCI

Equipment
- DES 3.0x18 mm 0 on hand · DEVICES IF PCI lists "DES 3.0x18 mm" · STOCK: "no resupply scheduled"

Relay: Imaging → Equipment, Labs → Meds.

Summary and approvals

Ruling recorded	none yet
Flags	Imaging 3 · Labs 2 · Meds 1 · Equipment 1
Rules that fired	Likelihood band (proposed rule, 9 points): higher. A label from a points rule, not a probability. Chair to decide; the packet lists equipment for both branches below.

For the chair to decide / check (the agents only flag):

- The likelihood points rule is proposed: the chair verifies it.
- The packet lists equipment for both branches: confirm stock for the branch actually taken.
- Thresholds in thresholds.json are proposed: the chair verifies them before this plan is used.
- Template logic (likelihood weights and bands, prep rules, Imaging triggers, Heart Team factors, stage assignment, Labs' eGFR cutoff) is unreviewed.

Approvals

No approval decisions recorded yet.

For the chair's use: print and mark.

- ☐ Approve as drafted
- ☐ Chair changes the plan (record the change)
- ☐ Equipment issue noted

Status

This draft is not approved until the chair presses Allow once on the Switch card.

AWAITING CHAIR

Sources: packet (day2.md) · briefs from Imaging, Labs, Meds, Equipment · thresholds.json · evidence table docs/EVIDENCE.md (E1 to E16, mostly secondary summaries, chair verifies). Generated 2026-11-05 19:10 by plan.py (template, no model).
Provenance: template a0c1d013d2 · thresholds 51ec501c23 · plan.py d0e4e2d2e3 · packet id n/a. "ref:" on a flag names a published criterion; it is not an instruction, and the criteria are unverified until the chair has checked them against the sources (docs/EVIDENCE.md).